



Psychological Disorders 2

Major Classifications of Psychological Disorders: Mood Disorders

Lecture 17

Summer 2026

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- By the end of class, students should be able to...
 - ...Compare some varieties of depression, such as **Major Depressive Disorder**, **Dysthymia**, and **Double Depression**.
 - ...Give examples of some biological and experiential factors that increase the risk for depression.
 - ...Describe the **onset**, **prognosis**, **risk factors**, and **comorbidity** profile of **Major Depressive Disorder**.
 - ...Characterize and give an example of **Helplessness Theory**.
 - ...Differentiate **Bipolar Disorder** from Major Depressive Disorder, and discuss biological and psychological factors implicated in Bipolar Disorder.

Daily Objectives



Moods are emotional states that are long-lasting, non-specific, and tend to be less intense.

Mood

- What is **mood**?
 - Think back to our Emotions & Motivation Module!
- Emotional states that are...
 - Long-lasting.
 - Nonspecific.
- How are moods different from **emotions**?

- **Mood disorders** are some of the most well-known psychopathological disorders.
 - Mental disorders that have mood disturbance as their prominent feature.
 - The most common type is depression.
- **Manic Episode**
 - A distinct period of high energy and increased activity.
 - E.g., Present in Bipolar Disorder.
- **Depressive Episode**
 - A distinct period of sad mood and loss of interest or pleasure.
 - E.g., Major Depressive Disorder, Dysthymia, Double Depression.



Depiction of mania (left) and depressive (right) symptoms.

Mood Disorders

- **Depressive Disorders:** key features include feelings of sadness, emptiness, hopelessness and irritability.
 - Long-lasting symptoms that affect many aspects of an individuals' life.
- Present in 22% of the female Canadian population and 14% of the male Canadian population. Women are 2-3 times more likely to be diagnosed with depression. Why?
 - Hormonal differences.
 - Different coping strategies (including sharing and co-rumination).
- About 1 in 12 Canadians will experience major depression in their lives.



Depressive Disorders



- The most well-known depressive disorder is **Major Depressive Disorder**.
 - Also known as **Unipolar Depression**.
- “Severely depressed mood and/or inability to experience pleasure that lasts two or more weeks and is accompanied by feelings of worthlessness, lethargy, sleep disturbance, and/or appetite disturbance.” – DSM-5

Depressive Disorders

- Diagnostic Criteria:

1. Five or more of the following symptoms present during the same 2-week period:

- Depressed mood
- Diminished interest
- Significant weight loss/gain
- Insomnia or hypersomnia
- Psychomotor agitation or retardation
- Fatigue
- Feelings of worthlessness/guilt
- Diminished concentration/decisiveness
- Recurrent thoughts of death/suicidal ideation

Major Depressive Disorder (MDD)

- Diagnostic Criteria:

2. No evidence of a **manic** episode (abnormal, persistent high mood).
3. Symptoms cause clinically significant distress/impairment.
4. Not better described by another DSM disorder.
5. Not attributable to another medical condition or physiological effects of substance use.

Major Depressive Disorder (MDD)

- **Onset:**
 - May appear at any age, but is most likely to appear in the 20s.
- **Prognosis:**
 - 2/5 of individuals recover within 3 months.
 - 4/5 of individuals recover within 1 year.
 - 1/5 of individuals do not experience remission.

Major Depressive Disorder (MDD)

- **Risk Factors:**
 - Temperamental (particularly neuroticism, or *negative affect*).
 - Environmental (childhood experiences, stressful life events).
 - Biological (neurotransmitter imbalance).
 - Genetic (family members of individuals with MDD are 2-4 times more likely to be diagnosed with MDD; ~40% heritability).
- **Comorbidity:**
 - Substance-related disorders, panic disorders, obsessive-compulsive disorder, anorexia nervosa, bulimia nervosa.

Major Depressive Disorder (MDD)



- Psychological factors can affect an individual's susceptibility to depression.
- Many theorists argue that it is the way we think about things that cause depression, rather than the things themselves.

Major Depressive Disorder (MDD)

- The **Helplessness Theory** argues that the way a person thinks about failure makes him/her/they more or less likely to be depressed.
 - Attribute failures to **internal characteristics**.
 - Believe that failures are permanent (stable).
 - Believe that failures are global (apply to many areas of life).

Helplessness Theory

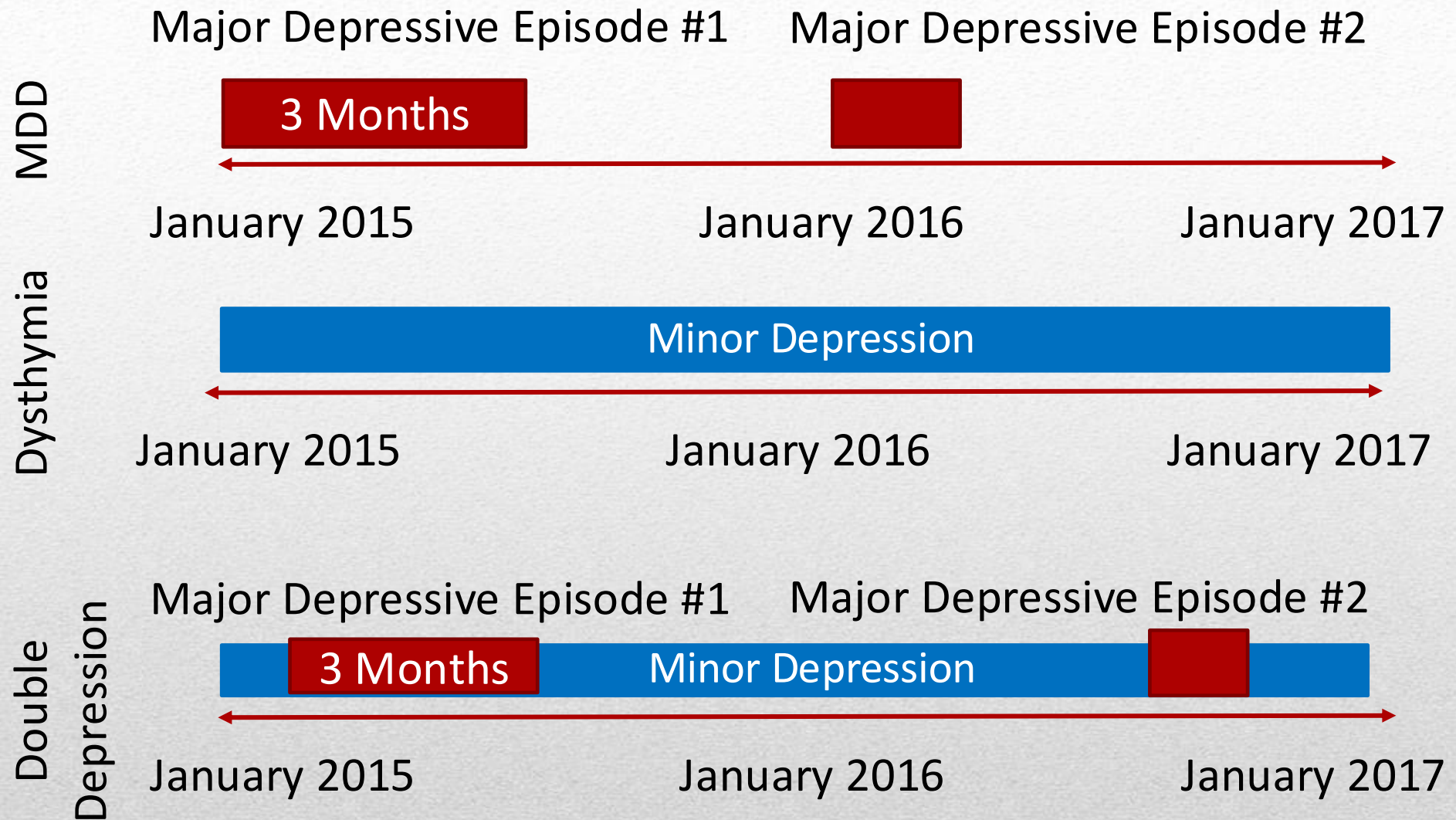
Hypothetical Situation:
My partner broke up with me.

	Helpless	Healthy
Internal vs. External	"I was a bad partner." "I'm not attractive." "I'm not good enough."	"It's not that I'm a bad person but the situation didn't work out."
Stable vs. Mutable	"I will always have luck like this in relationships." "I can't change who I am."	"Things can change. Things may be different next time."
Global vs. Local	"No one will ever love me again." "This is another one of many life failures."	"This particular relationship didn't work out."

Example of Helplessness Theory

- Sometimes depression lasts for a long time.
- Moderate depressive symptoms that last for more than two years are referred to as **Dysthymia** or **Dysthymic Disorder**.
- When dysthymia is punctuated by episodes of major depression, it is called **Double Depression**.

Depressive Disorders





Depiction of two sides of the mood spectrum.

- Mood disorders are not always **unipolar**.
- **Bipolar Disorders** are characterized by cycles of abnormal, persistent high mood (mania) and low mood (depression).
 - **Bipolar I Disorder**—at least one manic episode, possibly with hypomanic and depressive episodes as well.
 - **Bipolar II Disorder**—presence of both hypomanic and depressive episodes; no manic episodes.

Bipolar Disorders

- Diagnostic Criteria for Bipolar I Disorder:

1. Distinct period of abnormal, persistently elevated mood; increased activity or energy; lasting at least 1 week (manic episode).
2. Three or more of the following:
 - Inflated Self-Esteem
 - Decreased Need for Sleep
 - Talkative
 - Racing Thoughts
 - Distractibility
 - Increase in Goal-Directed Activity or Psychomotor Agitation
 - Excessive Involvement in Activities with a High Potential for Painful Consequences

Bipolar Disorder I

+ The Common
Three Criteria

3. Symptoms cause clinically significant distress/impairment.
4. Not better described by another DSM disorder.
5. Not attributable to another medical condition or physiological effects of substance use.

Bipolar Disorder I

- **Prevalence:**
 - 1 in 40 individuals
 - Not different between women and men (1 : 1)
- **Onset:**
 - Mean age of first episode = 18 years
 - Onset can occur for the first time in the 60s and 70s.
- **Prognosis:**
 - 90% of individuals who experience a manic episode will experience more of them throughout life.
 - Full remission is very rare.

Bipolar Disorder I

- **Risk Factors:**
 - Genetic (among the most heritable disorders; coincidence among identical twins = 40-70%).
 - Environmental (high stress, highly emotionally expressive family members, separation/divorce).
 - Psychological (high neuroticism, high conscientiousness).
- **Comorbidity:**
 - Anxiety Disorders
 - Substance Use Disorders
 - Attention Deficit Hyperactivity Disorder (ADHD)
 - Behavioural Disorders

Bipolar Disorder I

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